

Business Requirements Document

Ambulance handover operating model and dashboard requirements

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Scope assumption: The BRD is written for a small-to-medium hospital health centre context and can be adapted to local clinical governance, regulatory, and IT policies.

1. Objectives

- Reduce ambulance-to-clinical handover delay and make waiting time visible.
- Standardise the minimum clinical and operational information captured at handover.
- Clarify roles and escalation rules during routine and peak periods.
- Create auditable records for quality improvement and operational performance review.
- Improve patient and staff experience by reducing repeated questions and uncertainty.

2. Scope

Table 1. Scope definition.

In scope	Out of scope
Ambulance arrival logging, SBAR handover form, escalation thresholds, dashboard KPIs, SOP, training, feedback capture	Ambulance dispatch system replacement, full electronic medical record implementation, procurement of ambulance vehicles, clinical treatment protocol redesign

3. Functional Requirements

Table 2. Functional requirements.

ID	Requirement	Priority	Acceptance criteria
FR-01	Record ambulance arrival date/time, crew ID, patient identifier, presenting complaint, and receiving nurse.	Must	Every ambulance arrival has a timestamped log entry.
FR-02	Capture SBAR handover fields using a standard template.	Must	Template includes Situation, Background, Assessment, Recommendation, allergies, medication, vitals, and red flags.
FR-03	Trigger escalation when red flags or waiting thresholds are met.	Must	Escalation reason, time, and notified clinician are recorded.
FR-04	Provide weekly KPI dashboard for leadership review.	Must	Dashboard shows median handover time, target compliance, incomplete records, escalations, and feedback.
FR-05	Capture staff and patient/carer feedback after implementation.	Should	Monthly feedback summary includes theme, sentiment, and action owner.
FR-06	Maintain audit trail of handover corrections.	Should	Corrected entries preserve original value, reason, date, and correcting user.

4. Non-Functional Requirements

Table 3. Non-functional requirements.

Category	Requirement	Measure
Usability	Handover form must be usable during urgent, high-pressure arrival conditions.	Core fields completed within 8 minutes.
Availability	Paper fallback must exist if digital form is unavailable.	Fallback pack stocked and checked each shift.
Privacy	Patient-identifiable information must be handled under hospital privacy rules.	Access limited to authorised clinical/operations staff.
Reporting	KPIs must be understandable to clinical and non-clinical managers.	Dashboard includes definitions and traffic-light thresholds.
Training	Staff must receive short role-based training before go-live.	At least 90% of target staff trained.

5. Traceability Matrix

Table 4. Requirement-to-benefit traceability.

Requirement	Linked benefit	KPI
FR-01 Arrival log	Improved visibility and queue control	Arrivals logged within 1 minute
FR-02 SBAR template	Reduced missing information	Incomplete handover rate
FR-03 Escalation trigger	Faster response to clinical risk	Escalations documented within target
FR-04 Dashboard	Sustained management oversight	Weekly KPI review completed
FR-05 Feedback	Adoption and experience improvement	Staff satisfaction and patient/carer feedback

6. Key Risks and Controls

Table 5. Risk register summary.

Risk	Impact	Control
Staff bypass the form during peak pressure	Incomplete data and inconsistent handover	Keep form short, assign receiving nurse, audit weekly exceptions
Clinicians perceive dashboard as punitive	Low adoption	Frame KPIs as service improvement and show team-level trends
Digital access unavailable	Process interruption	Maintain paper fallback and retrospective entry rule
Feedback data is biased	Misleading benefits report	Collect feedback from crews, nurses, clinicians, and patients/carers